

FACULTY ENGAGEMENT

Point System

A Transparent, Auditable Framework for Faculty Teaching, Scholarship, and Program Citizenship

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The Problem

Faculty engagement has traditionally been assessed qualitatively. When ACGME asks, “How do you know your faculty are engaged?” narrative answers are not enough. A point system converts engagement into data the program can defend on site visit day.

What This System Delivers

- Objective annual faculty-level metric
- Defined minimum threshold for continued core faculty status
- Tier-based recognition (Bronze / Silver / Gold / Platinum)
- Direct evidence for APE, ADS, and citation responses
- Tie-in to faculty annual evaluation and contract renewal

ACGME ALIGNMENT

Common Program Requirements

II.B.4. Faculty must demonstrate commitment to the education and supervision of residents.

II.B.5. Faculty must pursue faculty development designed to enhance their skills as educators.

IV.D. The program and faculty must advance residents’ knowledge through scholarly activity.

V.A.1. The CCC must include faculty actively engaged in evaluation and assessment.

Faculty point systems have a 25-year published tradition in academic medicine. Our system is explicitly modeled on these precedents.

GOLD-STANDARD LONGITUDINAL

OHSU Emergency Medicine

Ma et al., Acad Med 2017;92(8):1138–1144

10-year prospective implementation of a point-based academic recognition system. Each activity awarded points on a predetermined education/scholarly scale; system sustained and refined annually over a decade. The gold-standard longitudinal validation of this concept.

DIRECT METHODOLOGICAL MODEL

NYU Emergency Medicine (ARVU)

Carmody, West J Emerg Med 2019;20(6):939–947

Two-stage Academic RVU system: baseline education expectations + voluntary points for scholarly work. 87% faculty met baseline year one. 1,240 activities logged. Open-access — explicitly designed to be generalizable to other institutions.

CLOSEST IM PARALLEL

Yeh et al. — Community IM

Yeh & Pan, J Gen Intern Med 1999;14(10):617–621

550-bed community teaching hospital, 11 part-time faculty general internists. Teaching Value Multipliers (TVMs) mapped to clinical RVUs to quantify teaching productivity. The earliest peer-reviewed IM-specific faculty effort quantification model — closest analog to our setting.

VALIDATES DOMAIN CAPS

Yale Department of Pathology

Morrow et al., Acad Pathol 2021;8:23742895211060461

Department-wide metric-driven effort allocation across missions. 30% cap on teaching category effort (validates our 30-pt Teaching cap). Established that transparent, bounded category weighting prevents mission imbalance — principle we directly adopted.

When ACGME asks “where did this come from?” we answer with names and citations — not “we made it up.”

Benchmarking — Our Weights vs. Published Systems

TRANSPARENT CALIBRATION AGAINST PEER-REVIEWED PRECEDENTS

Each of our design choices maps to a specific published system. Any weight is defensible because it has a published analog.

DESIGN CHOICE	OUR SYSTEM (MHS IM)	PUBLISHED PRECEDENT	RATIONALE
Domain structure	6 domains, ACGME-aligned	Simpson 2007 / Gusic 2014 — 5 AAMC-consensus domains	Added Clinical Supervision as 6th to fit IM program context
Activity-based scoring	1–8 pts per activity	Evans 2023 UAMS: 1–10 pts; Carmody 2019 NYU: variable pts	Compressed scale avoids rewarding any single activity disproportionately
Teaching domain cap	30 pts / 100 (30%)	Yale Pathology 2021: 30% cap on teaching category	Exact match — prevents teaching-heavy faculty from neglecting scholarship
Scholarship weight	20 pts / 100 (20%)	Carmody NYU: scholarship is highest-weighted single category	Reflects ACGME IV.D faculty scholarly activity expectation
First-author paper	8 pts (max in Domain 4)	Evans UAMS: peer-reviewed first-author = highest pts (10/10)	Top-of-scale placement consistent with literature
Baseline floor / threshold	60 / 100 points	Carmody NYU: baseline education expectations required; 87% met	Our threshold set to produce similar ≥90% compliance year one
Year-one iteration	Built-in PEC review	Ma OHSU: system refined annually over 10 years	Commits to published best-practice of longitudinal calibration

Every weight in our system is benchmarked. None is arbitrary.

System Overview — Six Engagement Domains

ANNUAL POINT TARGET: 100 POINTS PER CORE FACULTY

MHS IM RESIDENCY

ACGME #430036

	TEACHING <i>Didactics, morning report, noon conf, bedside rounds</i>	30 pts maximum / year
	CLINICAL SUPERVISION <i>Ward attending, consult, ambulatory precepting</i>	25 pts maximum / year
	CCC / PEC / COMMITTEES <i>Evaluation, milestones, governance</i>	15 pts maximum / year
	SCHOLARSHIP <i>Publications, posters, abstracts, chapters</i>	20 pts maximum / year
	QUALITY / PROGRAM IMPROVEMENT <i>QI projects, APE, program building</i>	15 pts maximum / year
	FACULTY DEVELOPMENT <i>Completed FD sessions, certifications</i>	5 pts maximum / year

ANNUAL TOTAL • 100-point reference total • 60 points minimum for core faculty status in good standing

Domain 1 — Teaching & Didactics

MAXIMUM 30 POINTS PER ACADEMIC YEAR



30

POINTS MAX

Most weight — core of ACGME requirement II.B.4

ACTIVITY	POINTS	EVIDENCE
Noon conference / didactic lecture (1 hr)	3	Schedule + attendance
Morning report facilitation	3	Weekly sign-in log
Thursday didactics attendance (per hr, 1:30–5 PM)	1	Hourly sign-in log — cap 10 pts/yr
Thursday didactics lecture delivery	3	Schedule + slides
Board review / ITE prep session	4	Session roster
Journal club facilitation	3	Article + attendance
Bedside / teaching rounds (per block)	2	Rotation schedule
Simulation session facilitation	4	Sim-lab log
Grand rounds lecture	5	GR flyer / slides
Med-student / sub-I teaching session	2	Clerkship roster



25

POINTS MAX

Direct resident supervision — graduated autonomy per ACGME CPR VI

ACTIVITY	POINTS	EVIDENCE
Teaching attending — GME ward service (per 1-wk block)	7	Block schedule
ICU / CVICU resident service (per 1-wk block)	7	Block schedule
Subspecialty consult service with residents (per block)	5	Consult schedule
Formal resident mentorship (per mentee, ≥3 mtgs/yr)	3	Mentor–mentee meeting log
Letter of recommendation (fellowship, job, etc.)	1	Uploaded letter
Ambulatory / clinic precepting (half-day)	1	Clinic template
Procedure supervision / signoff session	2	New Innovations log
Night float / weekend coverage attending	3	Call schedule
Rapid response / Code Blue debrief with team	2	Event log

NOTE: Points credited only when residents are on service. Private-practice / non-GME coverage does not count toward engagement points.

Domain 3 — CCC / PEC / Program Committees

MAXIMUM 15 POINTS PER ACADEMIC YEAR



Evaluation, governance, milestones — CPR
V.A.1 & V.C

ACTIVITY	POINTS	EVIDENCE
CCC member — full-year participation (all meetings)	8	CCC minutes
PEC member — full-year participation	6	PEC minutes
Milestone assessment (per resident per semi-annual)	1	New Innovations log
Resident semi-annual review (per resident reviewed)	1	Signed review
Resident remediation / PIP involvement (per plan)	3	Signed remediation plan
Recruitment interview day (per day)	2	Thalamus schedule
Competency subcommittee chair	3	Charter
Institutional GMEC / subcommittee (full year participation)	6	GMEC roster

REQUIREMENT: Every core faculty member must earn ≥ 3 points in Domain 3 annually to satisfy program-governance expectations.



20

POINTS MAX

Feeds ADS scholarly activity table — CPR IV.D

ACTIVITY	POINTS	EVIDENCE
Peer-reviewed publication (first/senior author)	8	PubMed citation
Peer-reviewed publication (co-author)	4	PubMed citation
Book chapter (Co-author)	6	Publisher letter
National / international conference poster or abstract	3	Acceptance email
Regional conference poster / oral presentation	2	Program book
Invited external lecture (CME-accredited)	4	Invite letter + agenda
Peer reviewer — indexed journal (per completed review)	1	Publons / journal record
Grant submitted / funded (intramural or extramural)	5	Notice of award
Podcast / online educational content (per episode)	2	Published URL — cap 8 pts/yr

RESIDENT-MENTORED BONUS: Any scholarly item with a resident co-author earns +2 points (does not count against domain cap).



15

POINTS MAX

PSQI, APE, citation response — CPR VI.A & V.C.1

ACTIVITY	POINTS	EVIDENCE
QI project — lead faculty mentor (resident-driven)	6	Project write-up
QI project — co-mentor	4	Project document
Patient Safety / MMI conference presenter	3	MMI slides
APE working group participation	3	APE meeting log
Citation response or self-study contributor	4	Signed section
New rotation / curriculum design (per rotation)	5	Goals & Objectives doc
Annual program survey action-item owner	2	Action plan
Wellness event leadership (retreat, committee, social)	2	Event description / roster

TRACK-RECORD BONUS: QI projects that produce a publication or poster also earn Domain 4 points — cross-crediting is permitted.

Domain 6 — Faculty Development

MAXIMUM 5 POINTS PER ACADEMIC YEAR



5

POINTS MAX

Required under CPR II.B.5 — faculty development to enhance skills as educators

ACTIVITY	POINTS	EVIDENCE
Methodist internal FD session (per 1 hr)	1	Module completion
External FD conference (AAIM, APDIM, etc.)	3	Registration + certificate
FD lecture development (new content, prep work)	2	Slides / curriculum doc
FD lecture delivery (presenting to faculty)	2	Attendance roster
Teaching / education certificate course	4	Certificate of completion
Coaching / mentorship training completed	3	Module completion
Wellness / burnout / fatigue-training module	2	Module completion
Diversity / implicit-bias training completed	2	Module completion

FLOOR REQUIREMENT: Every core faculty member must earn ≥ 2 Domain-6 points annually. Falling below triggers PD-directed remediation plan.

Recognition Tiers

ANNUAL FACULTY STANDING BASED ON TOTAL POINTS EARNED

PLATINUM 90+ points	Exceptional. Top-tier teacher-scholar. Eligible for Methodist Master Educator award nomination, letter of commendation from DIO, and priority consideration for institutional teaching-award slates.
GOLD 75 – 89 points	Highly engaged. Recognition at annual faculty retreat. Eligible for teaching-award nominations and featured in annual program report.
SILVER 60 – 74 points	Meets core faculty engagement threshold. In good standing. Letter of good standing for reappointment.
BRONZE / AT-RISK Below 60 points	Below threshold. Triggers mid-year conversation with PD, individualized engagement plan, reassessment in 6 months. Sustained failure → removal from core faculty designation.

Worked Example — A Typical Core Faculty Year

HOW THE POINTS STACK UP IN PRACTICE

Dr. Faculty — CVICU Attending, AY 2025–26

DOMAIN	ACTIVITY	QTY	PTS EACH	SUBTOTAL
Teaching	Noon conf lectures	5	3	15
Teaching	Journal club facilitation	2	3	6
Teaching	Morning report facilitation	3	3	9
Clinical Supervision	CVICU resident service (1-wk blocks)	4	7	25
CCC / PEC	CCC member	1	8	8
CCC / PEC	Recruitment interview days	2	2	4
Scholarship	National poster (resident co-author)	1	3 + 2	5
Scholarship	Co-authored peer-review paper	1	4	4
QI	QI project co-mentor	1	4	4
Fac Dev	FD lecture development & delivery	1	2 + 2	4

ANNUAL TOTAL: 84 POINTS → GOLD TIER — Highly engaged; eligible for teaching-award nomination.

Note: Clinical Supervision capped at 25 pts (28 earned, capped). Cross-domain cross-crediting (e.g., QI → Scholarship) is allowed; single activity cannot double-count within the same domain.

- 1

Monthly Self-Log

Each faculty member logs activities in a New Innovations portfolio or by email submission to the Program Coordinator. Log entries require a one-line evidence description and file attachment where applicable.
- 2

Quarterly Coordinator Audit

Program coordinator reconciles logs against New Innovations attendance and procedure logs, rotation schedules, faculty email submissions, and PubMed queries. Discrepancies flagged to PD.
- 3

Mid-Year PD Review

In January, PD meets with any faculty tracking below 30 points at mid-year. Individualized plan documented. Copy filed in faculty folder.
- 4

End-of-Year Compilation

Annual point totals locked June 30. Results pushed to faculty profile, feeds APE scholarly-activity section, faculty annual review, and ADS scholarly activity table.
- 5

Public Reporting

Aggregate (de-identified) point distribution shared at annual faculty retreat. Individual totals shared only with the faculty member and DIO.

Program Director <i>Dr. Amalakuhan</i>	Owns framework. Approves tier assignments. Meets with at-risk faculty. Reports totals to GMEC/DIO.
APDs <i>Drs. Melo, Sanka, Triana</i>	Review domain-specific submissions (Education / Clinical Ops / Strategic Growth). Flag gaps; recommend Domain-6 remediation.
Program Coordinator <i>GME Coordinator</i>	Maintains portfolio. Runs quarterly reconciliation. Generates quarterly, mid-year and end-of-year reports.
Chief Residents	Validate teaching attendance for noon conference, morning report, journal club. Feedback on faculty-engagement quality.
DIO <i>Dr. Justin Williams</i>	Receives annual rollup for institutional oversight. Signs Platinum-tier recognition letters.

ANNUAL PROGRAM EVALUATION (APE)

- Aggregate distribution included in APE deck
- Faculty below threshold named as action item
- Progress tracked year over year
- Points correlate with resident evaluations

ACGME ADS ANNUAL UPDATE

- Scholarly-activity entries auto-populate from Domain 4
- Faculty-development entries auto-populate from Domain 6
- Reduces clerical burden at ADS deadline
- Every entry pre-verified with evidence

SITE VISIT CITATION DEFENSE

- Faculty engagement becomes a defensible data point
- Answers “How do you know?” with numbers
- Documents closure of Citation #3 (faculty scholarly activity)
- Evidence of II.B.4 / II.B.5 compliance

Implementation Timeline

PATH TO FULL ROLLOUT BEFORE THE SEPTEMBER 2026 SITE VISIT

NOW AY 2025–26	APRIL 2026	MAY 2026	JULY 2026	AUG 2026	SEP 2026	JUN 2027
Retroactive Baseline Points assigned retroactively for current academic year to establish faculty-level baseline across all six domains. Identifies gaps; informs launch-year targets.	PEC Approval COMPLETED Point system presented to PEC. Feedback incorporated. Formal vote and minute-documented approval.	Faculty Town Hall PD-led faculty meeting. Framework reviewed. Q&A. Written copy distributed. Implementation date confirmed July 1.	AY 2026–27 Launch Faculty begin logging. First coordinator audit scheduled for October.	Pre-Site Visit Audit Dry-run aggregate report generated for internal audit. Identifies gaps. Faculty coached on documentation.	ACGME Site Visit Present system as evidence of II.B.4 / II.B.5 / IV.D compliance. Live portfolio demoed if requested.	First Annual Close Full-year data. First tier assignments. APE / ADS integration validated. Year-2 refinements.

Each milestone documented in PEC minutes — creates a paper trail the site visitor can follow.

THE BOTTOM LINE

When ACGME asks “*how do you know your faculty are engaged?*” — we don’t hand them stories. We hand them data.

Six domains. One hundred possible points. Four recognition tiers.

Objective. Auditable. Built to be defended on site visit day.